SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Capecitabine + temozolomide (CAPTEM)

Locally advanced or metastatic pancreatic neuroendocrine tumour under NCCP 00505a. · NCCP 00505 · version 5 · 28-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
CAPTEM combines two oral chemotherapy medicines to help control an advanced pancreatic NET. The team discusses the intended benefit, side effects and alternatives. NCCP identifies this pancreatic NET use as outside the medicines' licences, requiring the local consent process.

Schedule
Each cycle lasts 28 days: capecitabine twice daily on Days 1–14; add temozolomide once daily on Days 10–14; take neither anticancer medicine on Days 15–28. NCCP plans six cycles unless progression or unacceptable toxicity changes the plan.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Capecitabine
CHEMOTHERAPY TABLET

- ☐ Diarrhoea / dehydration
- ☐ Hand-foot syndrome
- ☐ Mouth soreness
- ☐ Low blood counts / infection
- ☐ Heart problems / chest pain
- ☐ Severe early toxicity with DPD deficiency

Temozolomide
CHEMOTHERAPY CAPSULE

- ☐ Nausea / vomiting
- ☐ Low blood counts and delayed marrow suppression
- ☐ Infections including PJP
- ☐ Liver injury, which may be delayed
- ☐ Fatigue, appetite changes and headache
- ☐ Rare serious blood disorders

Discuss the off-label pancreatic NET use, evidence and alternatives, DPD testing, two different oral schedules, nausea prevention on temozolomide days, blood and liver monitoring including after cycles, infection/PJP, fertility and pregnancy precautions.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00505 v5.

SACTCheck v0.84.37
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